Meeting card. Thursday 2026-11-05, 3:30 PM, El Paso. Luis, Fernando, Jesse.

The four questions, in order (the "?" slides; asked aloud, nothing printed on the slide)

1. **Cover (1):** "Fernando, what did you build, and how many centers were you running at the end?" Let him talk.
2. **Vendor contracts (3):** "Have you signed any RIS or billing vendors yet?" If Exa: likely pick; three centers; the four terms in the contract; Phase 2 decides.
3. **Owner numbers (17):** "What would each of you want to see on a Monday morning?" Write every answer down.
4. **If it were my money (23):** "What would you do differently this time, building it from scratch?" Do not fill the silence.

The five lines to land

1. **Four in five appeals win. Almost nobody files one.** 11.5% of 4.1 million denials appealed, 80.7% of those overturned. Medicare Advantage, all services, not an imaging number; the only audited national figure. Then: every denial nobody appeals is revenue left on the table.
2. **13 hours a week, about 40 requests, and 40% have someone working only on this. You have not hired that person yet.** AMA 2025, self-reported.
3. **95% of MRI prior auths are approved. A process that approves 95% of what it touches is one you automate, not one you staff.** CMS: 220 million provider hours over ten years, in the rule; a projection, not measured savings.
4. **22.3% cancelled versus 2.0% no-show, seventy percent with notice.** Backfill, not reminders. UC Irvine, one center; your numbers will differ, the pattern will not.
5. **5.1% to 3.8% with a text reminder. 1.3 points, not "25%."** Same study. Your RIS will do this. Let it.

If short on time, skip in this order

17, then **23** (keep its question for the Thank You), then **14** to one sentence, then **8 and 9** to their closing lines. Never skip 3, 4, 6, 11, 18, 21, 22 or 24. At 27:00 and not yet on 21: jump to **21, then 24.** Price at about 30:30; Thank You at 34:30.

Never say

- Luis is your RadAssist partner, or any arrangement of Luis's. "Luis uses software I built."
- Building, site, address, property, or what was there before. Only "your new center."
- Scale, platform, roll-up, Akumin, the lawsuit, Desert Imaging, "when you sold to private equity." If he raises it: listen, say little, move on.
- A vendor score or ranking, or any vendor price. Exa is "likely," never "decided"; "the pick is Phase 2."
- Any three-year total. Running costs: "near two thousand a month in vendors, about five with support, an estimate, with a cap."
- "It stays on the box." A Mac mini calling Claude or ChatGPT sends the data to the AI company; the BAA protects it; only a local model keeps it in-building.
- Headcount reduction or layoffs ("changes what your people do"); "Jesus"; "you open in June" as a fact.

The demo in four lines (slide 11)

1. Tabs open on the phone (cellular) and the laptop; banner checked: green is live, orange is simulated, say which.
2. **Maria:** order, needs auth, packet, approve and submit, advance 3 days, approved. **Robert:** same fast, advance twice, denied, review appeal, sign and submit, advance, overturned. Then the Owner's Monday report.
3. Say it first: no medical-necessity rules on the test server, so the approve or deny is simulated. Everyone waits for you on **Advance 3 days.**
4. Reload to reset (Cmd+R, or pull down); Start over does not. Reload before the meeting and before handing the phone over.

The twelve answers, one line each

1. "**You're one guy. Why trust you?**" Luis uses my software on real patients every day, you'll click a working demo in a few minutes, and every phase is a fixed fee you sign off, so if I don't deliver Phase 1 you've spent seven thousand, not forty-five.
2. "**Hit by a bus. Who fixes it at 7 a.m.?**" Scheduling runs in the system you buy, with that vendor's support line; if my piece goes down your staff do it by hand; and you own the code, the specification and a runbook that says who to call.
3. "**$45k before a single patient. Why now?**" You're spending seven for Phase 1, then you decide; the data terms are only free before you sign a RIS or PACS vendor, and fixing them later means renegotiating or rebuilding.
4. "**If AI writes the code, why not a kid from UTEP?**" Anyone can buy the scanner; you pay for the person who knows what the image means: what the center needs on day one, which terms protect your data, what Texas law requires, and when the AI is confidently wrong.
5. **Jesse:** "**Where does patient data live, who sees it, the AI company?**" In the United States, always; by role, every view logged, your own isolated instance with its own database and keys; the AI company only under a signed business associate agreement, never kept, never trained on; a human signs anything clinical.
6. "**What if in March it takes longer and costs more?**" The price is fixed: fourteen stays fourteen; if the date moves the timeline moves, not the price; anything new is quoted and approved in writing before I touch it.
7. "**Why fixed fee? Did you pad it?**" With hourly every surprise is your cost, with fixed it's mine; each phase is priced separately with its deliverables listed, so you see exactly what seven buys, and you can walk after it.
8. "**Luis is your neighbor. How do we keep it clean?**" On paper: a written contract, fixed scope, a price per phase, your sign-off before each one; Luis isn't the referee, you and I deal directly, and you can stop after any phase. Never "RadAssist partner."
9. "**What do you need, and what do we get from Phase 1?**" A few hours with all three of you, your services, volumes and payer mix, and the opening date; six weeks later, five documents you own: requirements, the patient-flow map, the contract terms, a make-or-buy for each requirement with a running cost for anything built, and the shortlist with criteria. Four at signing, three on delivery.
10. "**What will it cost to run?**" Not in the forty-five: accounts in your name, at cost, no markup; a ballpark in Phase 1 and three ways to host what I build (your cloud, hybrid, on-site), each with a 3-year cost, in Phase 3; vendors expected near two thousand a month, about five thousand with the support plan, with a cap I can't pass without your OK.
11. "**What happens after opening?**" A support plan, a flat monthly fee for a set number of hours so you always know who to call and what it costs; or a role on your team, which we can talk about. (Three thousand a month for up to twenty hours, one-fifty beyond, from June, twelve months, ninety-day check-in.)
12. "**One sentence. Why say yes today?**" Because the decisions that are free today stop being free the day you sign your first vendor contract, and finding out if I'm right costs you one phase, not the whole engagement.

Three more, in one breath. "Can you guarantee the opening date?" No one honestly can; every date is conditional on the assumptions slide (appendix 35); what I guarantee is the price. "What if Luis leaves?" The contract is with the center; you own everything I build; nothing depends on one person, including me. "Who do you contract with?" I'll contract with whichever company runs operations, and the practice's patient data stays the practice's.

Scorecard (slide 18): "Eight candidates. I'd build five, buy two, and integrate the voice system." "The right-hand column is what each one actually does. Take a second with it." Quiet 15 to 20 seconds. Then the five builds, one line each. Close: "Any of these you'd push back on?" Stop talking.

Owner numbers (slide 17): one system, two views, and all three of you see both. The revenue page is the practice's; the operations page is everyone's. **Records (slide 16):** the clock in (HIPAA 30 days, Texas 15 business days); the QR-coded envelope label out, Luis's idea, say so; the exchange between facilities is still manual nationwide; I automate everything on our side of it.

Ask Luis before the meeting: (a) who signs for the operating company, and does the December 18 capital date affect the $4,000 retainer; (b) "Dr. Luis Ramos" or "Luis Ramos-Duran, M.D." on the cover, and "J. Fernando Escarzaga"?; (c) is "Quantum Radiology" the public name; (d) does the practice bill both the technical and the professional side of each scan, so every denial lands on the practice?